VEG | ER for Pets

Visit Summary

Visit Date: March 04, 2026 05:38 PM

Dr. Sarah Emond

Miami
2707 Biscayne Boulevard
Miami, FL 33137
786-539-0339

Oreo (Wang)
D.O.B.: 03/04/2019 | 5.7kg | 7y 0m | Cat | Ragdoll | FS
Patient ID: 3o93nl6c

Status: Discharged
Discharged Date: March 05, 2026 11:31 PM

CUSTOMER	PRIMARY RDVM
Shaochen Wang 612-391-6729 vincent.sc.wang@gmail.com	Goodvets Wynwood Office: 305-942-9100 wynwood@goodvets.com

Deceased Deceased Date: 03/05/2026

PRESENTING COMPLAINTS

Renal azotemia

HISTORY

Oreo, a 7 yo FS Ragdoll presents as a transfer from Goodvets Wynwood for severe azotemia.

Initial presentation to rDVM for hematemesis on saturday (5 days ago). She was treated with supportive care with no improvement.

Oreo re-presented to rDVM today for **lethargy and continued inappetence**. Physical exam was unremarkable. Bloodwork revealed **severe azotemia consistent with acute kidney injury** (BUN >130, creatinine above readable range, SDMA 43) with **marked hyperphosphatemia and severe hyperkalemia**. Urinalysis showed **inadequately concentrated urine (USG 1.018) with proteinuria, glucosuria, and hemoglobin/occult blood without evidence of infection (no bacteria or pyuria)**. Findings were concerning for **severe AKI**, and transfer for hospitalization and further management was recommended.

She is previously healthy, full BW 6 mo ago was WNL per O.

VITALS

SYSTEM	INTAKE	LAST RECORDED
Mentation	QAR	Dull/Obtunded
Mucous Membranes	Light Pink	Pink
Heart Rate	130 BPM	80 BPM
Capillary Refill Time	> 2 Seconds	1-2 Seconds

SYSTEM	INTAKE	LAST RECORDED
Respiratory Effort	normal	0
Respiratory Rate	24 BPM	28 BPM
Temperature	97.4 °F	98 °F
Weight	5.12 KG	5.7 KG

INITIAL PHYSICAL EXAM

SYSTEM	INTAKE	NOTES
Abdomen	Normal	
Ears	Normal	
Eyes	Normal	
Heart	**Abnormal**	Bradycardic (130 bpm), gallop rhythm. NMA.
Integument	Normal	

SYSTEM	INTAKE	NOTES
Lungs	Normal	
Lymph nodes	Normal	
Mouth	Normal	
Musculoskeletal	Normal	
Neurologic	**Abnormal**	QDR, ambulatory without ataxia or paresis. No cranial nerve or placing deficits.
Nose	Normal	
Pulses	Normal	
Rectal	Normal	
Urogenital	Normal	

Intake Note:

PROBLEMS AND DIFFERENTIALS

PROBLEMS	DIFFERENTIALS
Renal azotemia	

NOTES

March 04, 2026 at 06:00 PM

Progress Note

Dr. Bush presentation - 4 AM

problem list and differential diagnoses
--**Severe azotemia (BUN >130, creatinine 17.6)** r/o acute kidney injury secondary to: nephrotoxin exposure (lilies, ethylene glycol, NSAIDs, medications), acute tubular necrosis, ischemic renal injury, pyelonephritis, ureteral obstruction, severe dehydration; acute-on-chronic kidney disease
--**Severe hyperkalemia with bradycardia** r/o acute kidney injury with impaired potassium excretion, urinary obstruction, severe metabolic acidosis, tissue necrosis, Addison's disease
--**ECG changes and ventricular premature complexes** r/o secondary to hyperkalemia, secondary to renal disease, underlying cardiac disease
--**Severe metabolic acidosis (pH ~7.0, HCO_3 6.4)** r/o uremic acidosis secondary to renal failure, lactic acidosis, toxin exposure, severe systemic illness
--**Hyponatremia** r/o renal disease with electrolyte derangement, gastrointestinal losses, severe systemic illness, Addison's disease
--**Hyperphosphatemia** r/o acute kidney injury, decreased renal excretion, severe renal dysfunction
--**Hypocalcemia** r/o renal disease, hyperphosphatemia-associated calcium shifts, pancreatitis, toxin exposure
--**Inadequately concentrated urine (USG 1.018) with proteinuria and glucosuria** r/o renal tubular dysfunction secondary to acute kidney injury, renal disease, stress hyperglycemia, early diabetes mellitus
--**hypothermia** r/o secondary to A1 vs other
--**H T bili** r/o IMHA, liver disease, other
--**cardiomegaly** r/o patient variant, HCM, DCM, other
--**Anorexia and vomiting** r/o uremia secondary to renal disease, pancreatitis, toxin ingestion, gastrointestinal disease
--**Loss of corticomedullary distinction bilaterally on POCUS** r/o acute kidney injury, chronic kidney disease, acute tubular necrosis, nephrotoxic injury

prognosis
--Guarded to poor given severe azotemia, electrolyte derangements, and metabolic acidosis. Prognosis dependent on response to aggressive supportive care and underlying etiology

recommendations
--Recommended full bloodwork, x-rays, and POCUS
--24-hour hospitalization
--Continuous ECG monitoring and electrolyte reassessment
--Serial bloodwork and blood gas monitoring
--Monitor clinical status and reassess

diagnostics
Preformed at rDVM
--**Urinalysis**: USG 1.018, proteinuria, glucosuria, hemoglobin/occult blood present, quiet sediment without bacteria

--**CBC**: Hematocrit 38%, WBC ~9k, mild lymphopenia and eosinopenia consistent with stress leukogram, platelets within normal limits
--**Chemistry**: BG 166, SDMA 43, creatinine markedly elevated (above analyzer range initially), BUN >130, phosphorus >16.1, calcium 7.2 (low), sodium 133 (low), potassium 8.7 (high), chloride 97 (low), total protein 6.7, tbili 1.9, amylase 2,167
--**radiographs w/ review**:
---1. The heterogenous small intestinal contents may reflect transiting ingesta, however nonobstructive foreign material cannot entirely be excluded. Underlying gastroenteritis secondary to dietary indiscretion, allergies/intolerances, inflammatory and infectious etiologies warrant consideration. Other disease processes such as pancreatitis or systemic abnormalities contributing to the patient's reported clinical signs without radiographic abnormalities cannot be excluded.
---2. There is no renomegaly or perinephric effusion to suggest radiographic evidence of acute nephritis, however this cannot entirely be dismissed and may require further imaging such as an abdominal ultrasound for further assessment.
---3. Equivocal cardiomegaly (incompletely evaluated).

at VeG
--**Chemistry recheck**: creatinine 17.6, BUN >130, phosphorus >16.1, sodium 141, potassium 9.0, pancreatic lipase normal
--**ECG**: tall tented T waves with heart rate 100–120 BPM initially; occasional ventricular premature complexes started around 11 PM --> progressed to intermittent singlets and occasional doublets
--**cPOCUS**: La:Ao 1.66 (H)
--**aPOCUS**: bilateral loss of corticomedullary distinction, no pyelectasai or ureteral distension, gallbladder wall edema/ scant FF surrounding GB, CVCCI 54% (H)
--**PCV/TS**: 35% / 7.8; plasma pale yellow and mildly icteric
--**Blood glucose**: 196
--**Lactate**: 0.8
--**Blood pressure**: 148 mmHg (LF), 164
--**CG8 blood gas**: severe metabolic acidosis (pH ~7.0, HCO_3 6.4) with critical hyperkalemia (K 8.2) and hyponatremia (Na 125); mild hypoglycemia, mild anemia, and decreased oxygenation noted

treatments
--Albuterol puff x2
--Calcium gluconate 0.5 ml/kg IV slow administration, redosed for persistent ECG changes & VPCs
--Maropitant 1 mg/kg IV
--Ondansetron 0.5 mg/kg IV
--1 ml/kg Dextrose IV
--Second IV catheter placed
--Fluid therapy: 0.45% saline 11 ml/hr (RER) and LRS 10 ml/hr (5% dehydration corr. over 24 hr)
--Active heat support

client communications cont.
--Discussed PE findings and possible differentials with owner. Discussed recommendations. O consented to plan.
--Explained that findings are consistent with severe kidney injury with life-threatening electrolyte abnormalities
--Discussed need for intensive monitoring and stabilization during the first 24 hours of hospitalization
--Discussed that prognosis is guarded to poor and depends heavily on response to therapy and underlying cause
--Explained that some patients recover with several days of hospitalization while others worsen despite treatment
--Discussed that if Oreo clinically deteriorates or fails to respond to treatment, quality of life discussions may be necessary

PLAN
Consider additional dextrose boluses and/or dextrose CRI for K management w/ recheck of electrolytes.
Consider additional albuterol puffs.
Consider antibiotics.
Consider saline agglutination test to r/o IMHA as cause of elevated tbili
Recommend AUS/echo.

Transferred care to Dr. Zackular 4 AM

Lilia Bush - March 04, 2026 06:32 PM

POCUS+ Ultrasound

Performed By: Dr. Lilia Bush

Patient Position:

Abnormal (2)

ABDOMINAL

Kidney
Abnormal: Left, Right

Left	Loss of corticomedullary junction, no pyelectasia or ureteral distension. No subcapsular edema.
Right	Loss of corticomedullary junction, no pyelectasia or ureteral distension. No subcapsular edema.

Normal (18)

ABDOMINAL

Peritoneal Effusion
Fluid Score: 0 / 4
None appreciated: Subxiphoid, Left Flank, Right Flank, Urinary Bladder, Umbilical

Gallbladder Wall Edema
None appreciated

Bladder
Unremarkable

Colon
Unremarkable

Spleen
Unremarkable: Spleen, Splenic Body

THORACIC

Pleural Effusion
None appreciated: Left, Right, Subxiphoid (Caudal)

B-Lines
None appreciated: Subxiphoid (Caudal), Left, Right

Glide Sign
Present: Left, Right

Not observed (3)

ABDOMINAL

Cvc Collapsibility

Cava Collapsibility Index

Spleen
Not assessed: Splenic Hilus

March 05, 2026 at 12:11 AM

Procedure Note

CG8+

Lilia Bush - March 05, 2026 12:35 AM

POCUS+ Ultrasound

Performed By: Dr. Lilia Bush

Patient Position:

Abnormal (4)

ABDOMINAL

Peritoneal Effusion
Fluid Score: 1 / 4
Present: Subxiphoid

Subxiphoid - Scant

Gallbladder Wall Edema
Present scant FF fluid surrounding gallbladder

Kidney
Abnormal: Right

Right see previous

CARDIAC

LA:Ao
Abnormal - Ratio 1.66

Normal (20)

ABDOMINAL

Peritoneal Effusion
None appreciated: Left Flank, Right Flank, Urinary Bladder, Umbilical

Cava Collapsibility Index
Assessed - 0.33 max / 0.15 min 54%

Bladder
Unremarkable

Colon
Unremarkable

Kidney
Unremarkable: Left

Left see previous

Spleen
Unremarkable: Spleen

THORACIC

Pleural Effusion
None appreciated: Left, Right, Subxiphoid (Caudal)

B-Lines
None appreciated: Subxiphoid (Caudal), Left, Right

Glide Sign
Present: Left, Right

CARDIAC

Pericardial Effusion
None appreciated

Cardiac Tamponade
None appreciated

Contractility
Normal

Not observed (4)

ABDOMINAL

Cvc Collapsibility

Spleen
Not assessed: Splenic Hilus, Splenic Body

CARDIAC

Lvedd

March 05, 2026 at 03:37 AM

Progress Note

PE: Quiet dull depressed but rousable mentation. Lateral/sternal, prefers to lay, Cranial nerves are intact. LP, MM. <2s CRT. low heart rate, no murmurs or arrhythmias noted. Normal respiratory rate and effort. Normal bronchovesicular sounds in all quadrants. Strong, synchronous femoral pulses. Abdomen comfortable on palpation. Able to sit up/take a step, can hold up head and move/crawl in cage,

Plan: continue all treatments as previously described.

Updates:
Temp still low despite heat support
Bp trending down, then up
Gave 2x albuterol puff
1ml/kg dextrose bolus diluted 1:4 over 15 mins
Called CC consults to see further recommendations
Gave 100mg/kg ca glu diluted 1:2 over 20 mins
Gave additional 1ml/kg dextrose bolus diluted 1:4 over 15 mins
Gave 2U Regular insulin
0.45% Saline 11 ml/hr added 5% dextrose
DC LRS when started Bicarb
Bicarb 14 ml + 70 ml D5W - running in 2 boluses of 7 ml bicarb + 35 ml D5W each over 3 hours for a total of 6 hours
Collected UA for Uculture- pending now in fridge
Enrofloxicin 5mg/kg IM once

Called o several times in am, gave updates about all recommendations moving forward. O agreed and paid for additional dx. Doing more intervention, still not out of the woods yet, could still pass away. DWO dialysis at LEAD ER. Quote 10-15k per tx, can call when IM is there after 8am if interested and if stable enough can set up a transfer to hospital if interested.

March 05, 2026 at 05:23 AM

Progress Note

Summary of Consultation Notes
• Patient is a dialysis candidate — dialysis should be initiated soon.
• Recommended diagnostics:
• Urine culture submission
• Abdominal ultrasound
• Heart rate: 90–110 bpm.
• Hyperkalemia management:
• Calcium gluconate: 100 mg/kg (≈1 mL/kg of 10% solution) given slowly IV.
• Insulin + dextrose therapy.
• Albuterol administration.
• Severe metabolic acidosis present.
• Bicarbonate therapy plan:
• Calculated HCO_3 deficit: 28.4 mEq.
• Replace half of the deficit → 14 mEq.
• Use 8.4% sodium bicarbonate (≈1 mEq/mL): give 14 mL.
• Dilution: dilute 5× with 70 mL D5W (or sterile water) to reduce hyperosmolarity.
• Administration: CRI 14 mL/hr for 6 hours.
• Decrease regular IV fluids during this infusion.
• Monitoring goals after infusion:
• Recheck blood gas.
• Target:
• HCO_3: ~14–15
• pH: >7.1
• Base excess: −5 to −7
• Antibiotics: start broad-spectrum coverage (enrofloxacin).
• Etiology consideration: possible nephrotoxin exposure.

Danyell Reuwer - March 05, 2026 06:05 AM

POCUS Ultrasound

Performed By:

Patient Position:

Abnormal (0)

Normal (7)

ABDOMINAL

Peritoneal Effusion
Fluid Score: 0 / 4
None appreciated: Subxiphoid, Left Flank, Right Flank, Urinary Bladder, Umbilical

THORACIC

Pleural Effusion
None appreciated: Left, Right

Not observed (0)

March 05, 2026 at 07:25 AM

Progress Note

Date/shift: 3/5/26 7a-7p

Doctor: Dr. Emond

Working Diagnosis:
Azotemia (creatinine 17.6, BUN > 130)
Hyperphosphatema
Hyperkalemia - static
Lethargy
Hypothermia - improving
Hypoglycemia - improving
Bradycardia - static
Metabolic acidosis
Peritoneal effusion

PE: Dull/obtunded, MM pale pink, tacky, CRT <2s, bradycardic on auscultation; lung sounds are clear, eupneic. comfortable abdominal palpation, bladder is soft and small.

Diagnostics:
Serial BP (mmHg): 90, 90, 90, 90, 92
Serial BG: 49, 59, 196, 156, 157, 268, 258, 227, 233
Full abdominal US: pending
Culture: results pending
CG8: see attached
Renal Panel: CREA 17.3, BUN >130, SDMA 40, Na 132, K 9.6, Cl 96
PCV 37%, 32%
TS 6, 5.6
Lactate 0.4, 0.7
Lytes: Na 137, 132; K 10, 9.6; Cl 99, 96

Current treatments:
0.45% Saline 11 ml/hr + 5% dextrose
Enrofloxacin 5 mg/kg IV q 24 hr
Cerenia 1 mg/kg IV q 24 hr
Ondansetron 0.5 mg/kg IV q 8 hr
Bicarb 14 ml + 70 ml D5W - running in 2 boluses of 7 ml bicarb + 35 ml D5W each over 3 hours for a total of 6 hours
NG Tube feedings

Plan:
Recheck bloodwork this AM and in 6 hours. Continue monitoring. Discuss transfer to speciality for continued care/ dialysis.

CC:
Discussed update with owner. Discussed that she remains dull and her energy levels have not improved. Discussed concerns for low heart rate, high potassium, and low BG. Discussed transfer to speciality - owner interested in pursing specialty and dialysis. Discussed that she is not stable enough for transfer, but plan for today is to stabilize as much as possible to get her ready for transfer. Discussed QOL and humane euthanasia due to grave prognosis. Owner would like to pursue continued treatments at this time. Discussed we will also be rechecking bloodwork and the ultrasonographer comes in at 2 PM. Owner understood.

Discussed low BG with owner. Discussed persistent low BG despite being on 5% dextrose is concerning, and we may need to increase the dextrose % indicating the need for a central line at this time. Discussed risks of central line placement.

Discussed we will also pull blood from the central line so we will not have to keep drawing blood from different limbs. Also discussed recommendation for NG tube placement. Discussed that placement of the central line will increase her hospitalization level from level 2 to level 3, and will be accounted for in the new estimate. Owner understood and agreed to plan.

Discussed central line placement and NG tube placement were successful and we will begin bolus feeding her. Discussed she became severely hypothermic during placement so she will need to warm up prior to the abdominal US. Owner understood.

Discussed recheck bloodwork revealed a mild improvement in the creatinine but it is still severely elevated and her potassium remains severely elevated. Discussed abdominal US will return in a few hours and that she does have free fluid in her abdomen. Discussed sample of fluid is yellow in color and could be related to liver dsyfunction vs kidney failure vs FIP vs other. Recommended path review of the fluid. Discussed pursing continued hospitalization overnight vs transfer to specialty vs QOL. Owner would like to wait for US report and path review for results.

Finances:
Approved 24 hour estimate - ends at 7 PM

March 05, 2026 at 10:04 AM

Procedure Note

CG8

March 05, 2026 at 04:13 PM

Procedure Note

CG8

March 05, 2026 at 07:07 PM

Progress Note

7pm - 7am Dr. Klipin
Problem list:
Azotemia (creatinine 17.6 -> 17.3, BUN 130)
Hyperphosphatema
Hyperkalemia - persistent
Peritoneal effusion
Decreased appetite
Lethargy
Hypothermia
Hypoglycemia - improving
Bradycardia

Physical Examination:
MM pale pink, CRT < 2 sec and tacky. Bradycardia. Clear bronchovesicular sounds. Eupneic. Abd soft, nonpainful. Central line in place. Quiet mentation.

Treatments and diagnostics:
0.45% Saline 11 ml/hr + 5% dextrose
LRS 10 ml/hr
Bicarb 14 ml + 70 ml D5W - running in 2 boluses of 7 ml bicarb + 35 ml D5W each over 3 hours for a total of 6 hours
Enrofloxacin 5 mg/kg IV q 24 hr
Cerenia 1 mg/kg IV q 24 hr
Ondansetron 0.5 mg/kg IV q 8 hr
NG Tube feedings

Progress Notes:
8p- Discussed clinical condition with the owner. Owner asked about euthanasia. Discussed euthanasia vs transfer to specialty. Discussed that cause for disease is unknown at this time. Owner is intereted in answers but also understands the pet's perceived suffering. Discussed with the owner that the cytology can take a while to come back but owner elects to wait for AUS before making decisions and is considering transfer vs euthanasia. Discussed necropsy.

9p- AUS Report: Large right kidney (nephritis, suspect chronic component). DDX include FIP vs lymphoma. FF indicative of likely renal failure. SI shows chronic enteropathy such as SI lymphoma vs IBD vs fungal disease.
Discussed AUS report with owner. FIP vs lymphoma most likely and prognosis at this late stage is grave.
Owner elects euthanasia at this time.

10p- The owners elect to be present for euthanasia. The process was discussed as giving a sedative first, then the euthanasia agent that will stop the heartbeat, followed by saline flush to clear the line. Owners confirmed they were ready to say goodbye and drugs were given IV in the aforementioned order. Cardiac auscultation confirmed absence of a heartbeat. PATIENT passed peacefully.
3 mLs Propofol IV + 3 mLs Euthasol IV.

March 05, 2026 at 08:58 PM

Progress Note

ECC CONSULT

March 05, 2026 at 11:32 PM

Progress Note

AUS Report

March 06, 2026 at 09:45 AM

Communication Note

Discussed cytology results with owner. Discussed the abdominal fluid was very aceullular and there were not enough cellular components to send out for a pathology review. Offer a fluid analysis send out test via idexx and informed owner this would return in 5-7 days. Owner elected to hold on the fluid analysis as the necropsy is scheduled to be performed.

TREATMENT SUMMARY

MEDICATION NAME	LAST ADMINISTERED	DOSE	ROUTE
Regular Insulin 100 units/mL Injectable	03/05/26 05:56 AM	2.00 unit	IV
Calcium Gluconate 10% Injectable (50 mL Bottle)	03/05/26 05:56 AM	538.00 mg	IV
Sodium Bicarbonate 84 mg/mL (8.4%) Injectable	03/05/26 06:53 AM	14.00 ml	IV
Enrofloxacin 22.7 mg/mL Injectable	03/05/26 07:37 AM	26.90 mg	IV
Dextrose 50% Injectable	03/05/26 10:07 AM	5.38 ml	IV
Albuterol 90 mcg Inhaled	03/05/26 10:12 AM	1.00 puff	Inhaled
Midazolam 5 mg/mL Injectable	03/05/26 12:00 PM	1.35 mg	IV
Maropitant 10 mg/mL Injectable	03/05/26 06:38 PM	5.12 mg	IV
Ondansetron 2 mg/mL Injectable	03/05/26 08:21 PM	2.56 mg	IV
Sodium PENTobarbital (Euthanasia) Inj	03/05/26 11:31 PM	3.00 ml	IV
Propofol 10 mg/mL Injectable	03/05/26 11:31 PM	30.00 mg	IV

DIAGNOSTIC	DATE	RESULT
Blood Glucose (BG) - Glucometer	03/04/26 06:33 PM	196
Electrolytes - Catalyst	03/04/26 06:55 PM	See attached
Renal (BUN/CREA/PHOS) Panel - Catalyst	03/04/26 06:55 PM	See attached
Pancreatic Lipase (PL) - Catalyst Test	03/04/26 06:55 PM	See attached
CG8+ Blood Gas - i-STAT	03/05/26 12:35 AM	metabolic acidosis
POCUS/POCUS+ Ultrasound	03/05/26 06:05 AM	None
Urine Culture*	03/05/26 06:57 AM	See attached
Radiographs - Single Image without Review	03/05/26 03:03 PM	https://www.idexximagebank.com/emailView?emailToken=29ea3b7f-68ad-475c-becd-6798798e749b

DIAGNOSTIC	DATE	RESULT
Blood Lactate - Lactometer	03/05/26 03:52 PM	0.7
Packed Cell Volume (PCV)	03/05/26 04:12 PM	32
Renal (BUN/CREA/PHOS) Panel + SDMA - Catalyst	03/05/26 04:13 PM	See attached
Electrolytes - Catalyst	03/05/26 04:13 PM	See attached
Total Solids (TS) - Refractometer	03/05/26 04:13 PM	5.6
CG8+ Blood Gas - i-STAT	03/05/26 04:14 PM	see notes
Urine Specific Gravity (USG) - Refractometer	03/05/26 06:31 PM	1.016
VEG Sonographer - Abdominal Ultrasound	03/05/26 06:56 PM	see attached
Blood Glucose (BG) - Glucometer	03/05/26 08:00 PM	251
Blood Urea Nitrogen (BUN) - Catalyst	03/05/26 09:00 PM	See attached
Creatinine (CREA) - Catalyst	03/05/26 09:00 PM	See attached
Digital Cytology (Path Review) - Imagyst	03/05/26 09:49 PM	pending
AI Blood - Imagyst	03/05/26 09:50 PM	see results in zoetis

FLUID NAME	START/END	RATE (ML/HR)
Lactated Ringers (LRS)	03/04/26 07:00 PM 03/05/26 07:34 AM	10.0 ml / hr
0.45% Saline	03/04/26 07:00 PM 03/05/26 09:49 PM	11.0 ml / hr

Treatment Sheet

Oreo (Shaochen) | #3o93nl6c | 7y 0m, FS, Cat

VEG - Miami
786-539-0339
Dr. Sarah Emond

Wednesday, Mar 4, 2026 (Day 1 of 2)

ORDER	12AM	1AM	2AM	3AM	4AM	5AM	6AM	7AM	8AM	9AM	10AM	11AM	12PM	1PM	2PM	3PM	4PM	5PM	6PM	7PM	8PM	9PM	10PM	11PM
VITALS																								
Weight																			5.12 kg					
Temperature																			97.4 F IC		97.5 F 98.4 F		99.4 F 99.7 F	
Heart Rate																			130 BF IC	IC	120 BPM		112 BPM	11 3 BP
Respiratory Rate																								24 BPM
Respiratory Effort																			normal					0
Check Mucous Membranes (MM)																			Light Pin					Light Pi
Note Capillary Refill Time (CRT)																			> 2 Seco					> 2 Sec
Check Mentation																			QAR					QAR
MONITORING																								
Note Defecation																								
Note Vomiting/Regurgitation																								
Note Urination / Straining																								
Electrocardiogram (ECG)																			IC					
Blood Pressure (BP)																								
DIAGNOSTICS																								
Blood Glucose (BG) - Glucometer																								
Blood Lactate - Lactometer																								
Renal (BUN/CREA/PHOS) Panel - ...																					DISCONTINUED 03/04/202...			
Electrolytes - Catalyst																					DISCONTINUED 03/04/202...			
POCUS/POCUS+ Ultrasound																								

Notes IC Incomplete Complete

Treatment Details: To view details, please visit https://www.dogbyte.vet/treatment_details/jsr5xf

Treatment Sheet

Oreo (Shaochen) | #3o93nl6c | 7y 0m, FS, Cat

VEG - Miami
786-539-0339
Dr. Sarah Emond

Wednesday, Mar 4, 2026 (Day 1 of 2) cont.

ORDER	12AM	1AM	2AM	3AM	4AM	5AM	6AM	7AM	8AM	9AM	10AM	11AM	12PM	1PM	2PM	3PM	4PM	5PM	6PM	7PM	8PM	9PM	10PM	11PM
DIAGNOSTICS																								
Packed Cell Volume (PCV)																								
Total Solids (TS) - Refractometer																								
Pancreatic Lipase (PL) - Catalyst T...																								
MEDICATIONS																								
Albuterol 90 mcg Inhaled 1.0puff: None: Inhaled: once																								
Calcium Gluconate 10% Injectable ... 260.0mg: 2.6 ml: IV: once																								
Maropitant 10 mg/mL Injectable 5.12mg: 0.51 ml: IV: q24hr																								
Ondansetron 2 mg/mL Injectable 2.56mg: 1.28 ml: IV: q8hr																								
CRI, FLUIDS, O2																								
Lactated Ringers (LRS)																								
0.45% Saline																								
TASKS																								
Change Bedding																								
Walk Patient / Clean Cage																								
Offer / Refill Water																								
Place Peripheral IV Catheter																								
IV Catheter Breakdown / Change B...																			IC					
IV Catheter Care / Maintenance																			IC					

Notes IC Incomplete Complete

Treatment Details: To view details, please visit https://www.dogbyte.vet/treatment_details/jsr5xf

Treatment Sheet

Oreo (Shaochen) | #3o93nl6c | 7y 0m, FS, Cat

VEG - Miami
786-539-0339
Dr. Sarah Emond

Wednesday, Mar 4, 2026 (Day 1 of 2) cont.

ORDER	12AM	1AM	2AM	3AM	4AM	5AM	6AM	7AM	8AM	9AM	10AM	11AM	12PM	1PM	2PM	3PM	4PM	5PM	6PM	7PM	8PM	9PM	10PM	11PM
TASKS																								
Dr Updates Customer																								
Miscellaneous Task																								
Heat Support																								IC

Notes IC Incomplete Complete **Treatment Details**: To view details, please visit https://www.dogbyte.vet/treatment_details/jsr5xf

Treatment Sheet

Oreo (Shaochen) | #3o93nl6c | 7y 0m, FS, Cat

VEG - Miami
786-539-0339
Dr. Sarah Emond

Thursday, Mar 5, 2026 (Day 2 of 2)

ORDER	12AM	1AM	2AM	3AM	4AM	5AM	6AM	7AM	8AM	9AM	10AM	11AM	12PM	1PM	2PM	3PM	4PM	5PM	6PM	7PM	8PM	9PM	10PM	11PM
VITALS																								
Weight			5.38 kg						IC							5.7 kg							IC	...
Temperature	98.3 F		98.6 F	98.3 98.1 F		99.3 F	98.9 F		97.7 F	98.4 F	98.0 F 98.2 F	IC	95.7 F	IC	94.2 F	96.4 F	97.5 F 98.1 F		98.2 F		98 F	IC	IC	...
Heart Rate		121 BPM	104 BPM	120 100 BPM		112 BPM	100 BPM	104 BF 100 BPM		96 BPM	82 BP 72 BPM	IC	IC	IC	83 BPM		72 BPM 108 B 108 BPM		96 96 BPM		80 BPM	IC	IC	...
Respiratory Rate		32 BPM		20 BPM			40 BPM	20 BPM		18 BPM		IC		IC			30 BPM		36 BPM		28 BPM		IC	...
Respiratory Effort		0		0			1	0		0		IC		IC			0		0		0		IC	...
Check Mucous Membranes (MM)							Pink						IC				Pink						IC	...
Note Capillary Refill Time (CRT)							> 2 Seco						IC				1-2 Seco						IC	...
Check Mentation							Dull/Obtu						IC				Dull/Obtu						IC	...
MONITORING																								
Note Defecation																							IC	...
Note Vomiting/Regurgitation																							IC	...
Note Urination / Straining																							IC	...
Electrocardiogram (ECG)												IC	IC	IC	IC							IC	IC	...
Quantify Nasogastric (NG) Tube Fl...																							IC	
Blood Pressure (BP)								IC	IC			IC	IC	IC	IC			IC				IC	IC	...
DIAGNOSTICS																								
Blood Glucose (BG) - Glucometer								IC	IC						IC				IC		IC			DISCO...
Urine Culture* (4035)																								
VEG Sonographer - Abdominal Ultr...																								DISCO...
AI Blood - Imagyst																								DISCO...

Notes IC Incomplete Complete

Treatment Details: To view details, please visit https://www.dogbyte.vet/treatment_details/jsr5xf

Treatment Sheet

Oreo (Shaochen) | #3o93nl6c | 7y 0m, FS, Cat

VEG - Miami
786-539-0339
Dr. Sarah Emond

Thursday, Mar 5, 2026 (Day 2 of 2) cont.

ORDER	12AM	1AM	2AM	3AM	4AM	5AM	6AM	7AM	8AM	9AM	10AM	11AM	12PM	1PM	2PM	3PM	4PM	5PM	6PM	7PM	8PM	9PM	10PM	11PM
DIAGNOSTICS																								
Packed Cell Volume (PCV)																								DISCO...
Total Solids (TS) - Refractometer																								DISCO...
Blood Glucose (BG) - Glucometer									IC		DISCONTINUED 03/05/2026 9:59AM													
Blood Lactate - Lactometer																								DISCO...
Renal (BUN/CREA/PHOS) Panel - ...	DISCONTINUED 03/04/2026 7:46PM																							
Electrolytes - Catalyst	DISCONTINUED 03/04/2026 7:46PM																							
Electrolytes - Catalyst																								DISCO...
POCUS/POCUS+ Ultrasound																								DISCO...
Renal (BUN/CREA/PHOS) Panel + ...																								DISCO...
CG8+ Blood Gas - i-STAT																								DISCO...
CG8+ Blood Gas - i-STAT																								DISCO...
Blood Urea Nitrogen (BUN) - Catal...																								DISCO...
Creatinine (CREA) - Catalyst																								DISCO...
Radiographs - Single Image witho...																								DISCO...
Urine Specific Gravity (USG) - Refr...																								DISCO...
Pancreatic Lipase (PL) - Catalyst T...																	DISCONTINUED 03/05/2026 3:34PM							
Digital Cytology (Path Review) - Im...																								DISCO...
MEDICATIONS																								
Calcium Gluconate 10% Injectable ... 260.0mg: 2.6 ml: IV: once																				DISCONTINUED 03/05/2026 7:...				

Notes IC Incomplete Complete **Treatment Details**: To view details, please visit https://www.dogbyte.vet/treatment_details/jsr5xf

Treatment Sheet

Oreo (Shaochen) | #3o93nl6c | 7y 0m, FS, Cat

VEG - Miami
786-539-0339
Dr. Sarah Emond

Thursday, Mar 5, 2026 (Day 2 of 2) cont.

ORDER	12AM	1AM	2AM	3AM	4AM	5AM	6AM	7AM	8AM	9AM	10AM	11AM	12PM	1PM	2PM	3PM	4PM	5PM	6PM	7PM	8PM	9PM	10PM	11PM
MEDICATIONS																								
Albuterol 90 mcg Inhaled 1.0puff: None: Inhaled: once																				**DISCONTINUED 03/05/2026 7:...**				
Regular Insulin 100 units/mL Inject... 2.0unit: None: IV: once																				**DISCONTINUED 03/05/2026 7:...**				
Calcium Gluconate 10% Injectable ... 538.0mg: 5.38 ml: IV: once																				**DISCONTINUED 03/05/2026 7:...**				
Sodium Bicarbonate 84 mg/mL (8.... 14.0ml: 0.17 micro-ml: IV: once																				**DISCONTINUED 03/05/2026 7:...**				
Dextrose 50% Injectable 5.0ml: 0.01 micro-ml: IV: once																				**DISCONTINUED 03/05/2026 7:...**				
Dextrose 50% Injectable 5.38ml: 0.01 micro-ml: IV: once																				**DISCONTINUED 03/05/2026 7:...**				
Enrofloxacin 22.7 mg/mL Injectable 26.9mg: 1.19 ml: IV: q24hr																								
Sodium PENTobarbital (Euthanasia... 3.0ml: None: IV: once																								
Maropitant 10 mg/mL Injectable 5.12mg: 0.51 ml: IV: q24hr																								...
Midazolam 5 mg/mL Injectable 1.35mg: 0.27 ml: IV: once																				**DISCONTINUED 03/05/2026 7:...**				
Ondansetron 2 mg/mL Injectable 2.56mg: 1.28 ml: IV: q8hr																								...
Propofol 10 mg/mL Injectable 30.0mg: 3.0 ml: IV: once																								

Notes IC Incomplete Complete **Treatment Details**: To view details, please visit https://www.dogbyte.vet/treatment_details/jsr5xf

Treatment Sheet

Oreo (Shaochen) | #3o93nl6c | 7y 0m, FS, Cat

VEG - Miami
786-539-0339
Dr. Sarah Emond

Thursday, Mar 5, 2026 (Day 2 of 2) cont.

ORDER	12AM	1AM	2AM	3AM	4AM	5AM	6AM	7AM	8AM	9AM	10AM	11AM	12PM	1PM	2PM	3PM	4PM	5PM	6PM	7PM	8PM	9PM	10PM	11PM
CRI, FLUIDS, O2																								
Lactated Ringers (LRS)														DISCONTINUED 03/05/2026 7:34AM										
0.45% Saline																							DISCONTIN...	
TASKS																								
Administer IV Fluid Bolus																		DISCONTINUED 03/05/2026 3:35PM						
Inpatient Exam / Update																								...
Change Bedding																							IC	...
Walk Patient / Clean Cage																							IC	...
Feed Patient / Offer Food												IC											IC	...
Offer / Refill Water												IC											IC	...
Place Peripheral IV Catheter																		DISCONTINUED 03/05/2026 3:35PM						
Place IV Catheter (Central Line)																		DISCONTINUED 03/05/2026 3:35PM						
IV Catheter Breakdown / Change B...																							IC	...
IV Catheter Care / Maintenance																							IC	...
Place Nasogastric (NG) Tube																		DISCONTINUED 03/05/2026 3:35PM						
Check Nasogastric Tube (NG) Plac...																							IC	
Aspirate Nasogastric (NG) Tube																							IC	
Dr Updates Customer																								...
Miscellaneous Task																		DISCONTINUED 03/05/2026 3:36PM						
Miscellaneous Task																		DISCONTINUED 03/05/2026 3:36PM						
Heat Support												IC	IC	IC								IC	IC	IC

Notes IC Incomplete Complete **Treatment Details**: To view details, please visit https://www.dogbyte.vet/treatment_details/jsr5xf

Treatment Sheet

Oreo (Shaochen) | #3o93nl6c | 7y 0m, FS, Cat

VEG - Miami
786-539-0339
Dr. Sarah Emond

Thursday, Mar 5, 2026 (Day 2 of 2) cont.

ORDER	12AM	1AM	2AM	3AM	4AM	5AM	6AM	7AM	8AM	9AM	10AM	11AM	12PM	1PM	2PM	3PM	4PM	5PM	6PM	7PM	8PM	9PM	10PM	11PM
TASKS																								
Rotate / Turn Patient & Note Positi...																								IC
Trickle Feed Nasogastric (NG) Tube																							IC	
Lubricate (Lube) Eyes																								IC
Miscellaneous Task																								

Notes IC Incomplete Complete **Treatment Details**: To view details, please visit https://www.dogbyte.vet/treatment_details/jsr5xf

VEG | ER for Pets

Discharge Summary

Visit Date: March 04, 2026 05:38 PM

Dr. Sarah Emond

Miami
2707 Biscayne Boulevard
Miami, FL 33137
786-539-0339

Oreo (Wang)
D.O.B.: 03/04/2019 | 5.7kg | 7y 0m | Cat | Ragdoll | FS
Patient ID: 3o93nl6c

Status: Discharged
Discharged Date: March 05, 2026 11:31 PM

CUSTOMER	PRIMARY RDVM
Shaochen Wang 612-391-6729 vincent.sc.wang@gmail.com	**Goodvets Wynwood** Office: 305-942-9100 wynwood@goodvets.com

HOME CARE AND MONITORING

We are sorry for the loss of your pet. Please do not hesitate to reach out with any questions or concerns.

Thank you for trusting Veterinary Emergency Group Miami. You can reach us by email at miami@veg.com or by phone at 786-539-0339.

Dr. Rebecca Klipin

1 888 433-9987

OREO WANG

PET OWNER:	**SHAOCHEN WANG**	**VEG MIAMI**		LAB ID:	
SPECIES:	Feline	2700 BISCAYNE BLVD		ORDER ID:	312459831
BREED:		MIAMI, Florida 33137		DATE OF RECEIPT:	**3/5/26**
GENDER:	Female Spayed	7865390339		DATE OF RESULT:	**3/5/26**
AGE:	7 Years	ACCOUNT #:			
PATIENT ID:	3o93nl6c	ATTENDING VET:	Rebecca Klipin		

IDEXX Services: **Catalyst One Chemistry Analyzer**

Chemistry

3/5/26	**9:01 PM**			**3/5/26** 4:54 PM	**3/5/26** 4:26 PM
TEST	RESULT	REFERENCE VALUE			
Creatinine	- --.--	0.8 - 2.4 mg/dL		**17.3**	- --.--
BUN	**>130**	**16 - 36 mg/dL**	H		**> 130**

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1 888 433-9987

OREO WANG

PET OWNER:	**SHAOCHEN WANG**	**VEG MIAMI**		LAB ID:	
SPECIES:	Feline	2700 BISCAYNE BLVD		ORDER ID:	312414446
BREED:		MIAMI, Florida 33137		DATE OF RECEIPT:	**3/5/26**
GENDER:	Female Spayed	7865390339		DATE OF RESULT:	**3/5/26**
AGE:	7 Years	ACCOUNT #:			
PATIENT ID:	3o93nl6c	ATTENDING VET:	Sarah Emond		

IDEXX Services: **Catalyst One Chemistry Analyzer**

Chemistry

3/5/26	**4:26 PM**			**3/5/26** 4:11 PM	**3/5/26** 9:55 AM
TEST	RESULT	REFERENCE VALUE			
IDEXX SDMA	a **40**	**0 - 14 µg/dL**	H	- --.--	
Creatinine	- --.--	0.8 - 2.4 mg/dL		- --.--	
BUN	**>130**	**16 - 36 mg/dL**	H	- --.--	
Calcium	8.6	7.8 - 11.3 mg/dL		- --.--	
Sodium	**132**	**150 - 165 mmol/L**	L	- --.--	**137**
Potassium	**9.6**	**3.5 - 5.8 mmol/L**	H	- --.--	**> 10.0**
Na: K Ratio	14				
Chloride	**96**	**112 - 129 mmol/L**	L	- --.--	**99**

a SDMA:

SDMA is increased, no CREA result: likely impaired GFR and kidney function. Recommended next step: evaluation of other renal function tests, and complete urinalysis. For information on recommended actions visit: www.idexx.com/sdmaalgorithm.

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OREO WANG

PET OWNER:	**SHAOCHEN WANG**	**VEG MIAMI**		LAB ID:	
SPECIES:	Feline	2700 BISCAYNE BLVD		ORDER ID:	312277812
BREED:		MIAMI, Florida 33137		DATE OF RECEIPT:	**3/4/26**
GENDER:	Female Spayed	7865390339		DATE OF RESULT:	**3/4/26**
AGE:	7 Years	ACCOUNT #:			
PATIENT ID:	3o93nl6c	ATTENDING VET:	Lilia Bush		

IDEXX Services: **Catalyst One Chemistry Analyzer**

Chemistry

3/4/26 **6:57 PM**

TEST	RESULT	REFERENCE VALUE	
Creatinine	- --.--	0.8 - 2.4 mg/dL	
BUN	**>130**	**16 - 36 mg/dL**	H
Phosphorus	**>16.1**	**3.1 - 7.5 mg/dL**	H
Sodium	**141**	**150 - 165 mmol/L**	L
Potassium	**9.0**	**3.5 - 5.8 mmol/L**	H
Na: K Ratio	16		
Chloride	**98**	**112 - 129 mmol/L**	L
Catalyst Pancreatic Lipase	2.9	0.0 - 4.4 U/L	

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Veterinary Emergency Group
2707 Biscayne Blvd, Miami
Florida, 33137-4520, United States

Powered by ZoetisDx

Patient: - oreo wang, Cat, Unknown

Owner:

Requesting Doctor: None

i-STAT Alinity v - CG8+

Test	03/05/26 9:45 AM	Reference Interval	Graph
K	**8.6** mEq/L	3.4 - 4.9	High
Na	**123** mEq/L	145 - 157	Low
iCa	**1.22** mmol/L	1.04 - 1.44	
pH	**7.063**	7.28 - 7.46	Low
Glu	**29** mg/dL	70 - 161	Low
TCO2	**6** mEq/L	16 - 24	Low
pO2	**117** mmHg	90 - 110	High
pCO2	**19.9** mmHg	25 - 42	Low
BE	**-25** mEq/L	-11 - -1	
HCO3	**5.7** mEq/L	15 - 24	Low
sO2	**96** %	95 - 100	
Hb	**8.5** g/dL	9 - 17	Low
Hct	**25** %PCV	26 - 50	Low
Calculated pH	**7.067**		
Calculated PO2	**115** mmHg		
Calculated PCO2	**19.6** mmHg		

Veterinary Emergency Group
2707 Biscayne Blvd, Miami
Florida, 33137-4520, United States

Powered by ZoetisDx

Patient: - oreo wang, Cat, Unknown

Owner:

Requesting Doctor: None

i-STAT Alinity v - CG8+

Test	03/05/26 4:04 PM	Reference Interval	Graph
K	**7.6** mEq/L	3.4 - 4.9	High
Na	**122** mEq/L	145 - 157	Low
iCa	**1.08** mmol/L	1.04 - 1.44	
pH	**7.060**	7.28 - 7.46	Low
Glu	**201** mg/dL	70 - 161	High
TCO2	**11** mEq/L	16 - 24	Low
pO2	**65** mmHg	90 - 110	Low
pCO2	**34.0** mmHg	25 - 42	
BE	**-21** mEq/L	-11 - -1	
HCO3	**9.6** mEq/L	15 - 24	Low
sO2	**82** %	95 - 100	
Hb	**7.5** g/dL	9 - 17	Low
Hct	**22** %PCV	26 - 50	Low
Calculated pH	**7.075**		
Calculated PO2	**60** mmHg		
Calculated PCO2	**32.2** mmHg		

Radiology Services Report (12479125-15/Ultrasound Consult - Priority)

Completed 03/05/26 08:33 PM

Patient Name: Oreo Wang (3o93nl6c)

Requesting Doctor:	Dr. LILIA BUSH	**Species:**	Feline	**Age:**	7Yr 0Mo
Gender:	Female, Spayed	**Breed:**	Ragdoll	**Weight:**	5.70 Kg

Presenting Complaint: Ultrasound Consult

Organs of Interest: Sedation (no), US type (abdomen), Main abdominal organs of concern (general abdomen exam, urinary tract)

Patient History: Clinical signs summary (Oreo, a 7 yo FS Ragdoll presents as a transfer from Goodvets Wynwood for severe azotemia. Initial presentation to rDVM for hematemesis on saturday (5 days ago). She was treated with supportive care with no improvement. Oreo re-presented to rDVM today for lethargy and continued inappetence. Physical exam was unremarkable. Bloodwork revealed severe azotemia consistent with acute kidney injury (BUN >130, creatinine above readable range, SDMA 43) with marked hyperphosphatemia and severe hyperkalemia. Urinalysis showed inadequately concentrated urine (USG 1.018) with proteinuria, glucosuria, and hemoglobin/occult blood without evidence of infection (no bacteria or pyuria). Findings were concerning for severe AKI, and transfer for hospitalization and further management was recommended. She is previously healthy, full BW 6 mo ago was WNL per O.)

Physical Exam: Summary of PE findings (Bradycardic (130 bpm), gallop rhythm. NMA. QDR, ambulatory without ataxia or paresis. No cranial nerve or placing deficits.)

Diagnostics: Diagnostics (BG: 233 Lac: 0.7 PCV: 32% TP: 5.6)

Treatment: Tx plan (0.45% Saline 11 ml/hr + 5% dextrose LRS 10 ml/hr Bicarb 14 ml + 70 ml D5W - running in 2 boluses of 7 ml bicarb + 35 ml D5W each over 3 hours for a total of 6 hours)

Clinical Concern/Differentials/Impressions: Sonographer's impressions (Spleen: heterogenous, hypoechoic, slightly rounded edges. GB: moderately dilated, cystic duct appears distended. CBD: measures 0.39cm in one view, Kidneys: bilaterally show smooth renal capsule with mild loss of CMJ distinction, GIT: mild muscularis thickening noted throughout. One jejunal segment measures 0.21cm. AFS: 3/4, anechoic fluid mild. Left adrenal: measures 0.36cm), Main clinical question/concern (recommendation and next steps)

ABDOMINAL ULTRASOUND March 5, 2026: 22 still images and 15 cine loops are available for interpretation. Blood work results are also provided for correlation.

COMPARISON: None available.

Requested By:
Dr. LILIA BUSH
VEG MIAMI
2707 BISCAYNE BLVD
Miami, FL 33137
P: 786-539-0339
F:
F2:
re: Oreo Wang (3o93nl6c)

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Report provided by:
IDEXX Telemedicine Consultants
1-800-726-1212
9200 SE Sunnybrook Blvd., Suite 460
Clackamas, OR 97015

FINDINGS:

• Liver: The liver is normal in echogenicity and echotexture with adequate distinction of the portal markings and without evidence of nodules or masses.

• Gallbladder: The gallbladder is physiologically distended containing anechoic fluid and mild hyperechoic debris. The gallbladder wall is normal in thickness. The undulating proximal cystic duct measures 3.9 mm in diameter, but this is very close to the neck of the gallbladder, and appears much smaller, tapering to normal near the porta hepatis, measuring 1.7 mm in diameter, when calibrated to the scale along the right side of the image. This is not followed all the way to the duodenal papilla for this exam.

• Spleen: The spleen is normal in echogenicity and echotexture without evidence of nodules or masses.

• Pancreas: The left lobe of the pancreas caudal to the stomach is mildly hypoechoic to bright surrounding mesentery, normal in size, measuring only 4.6 mm thick. Images of the right pancreas are not provided

• Stomach: The stomach is nondistended containing echogenic fluid and gas. Gastric wall thickness is not provided but sonographic layering is normal near the pylorus, the fundus questionably fully provided on the study images. A normal pyloroduodenal junction is imaged.

• Duodenum: The labeled duodenum contains mild echogenic fluid and gas. Duodenal wall thickness is upper limits of normal for a cat at 3.4 mm and both the muscularis and submucosal layers appear similar in thickness to the mucosa.

• Jejunum: At least 1 imaged loop of bowel labeled jejunum is fluid-filled, measures upper limits of normal in diameter at 1.2 cm, when calibrated to the scale along the right side of the image. This loop is not measured for thickness, wall layering slightly indistinct, but additional loops have variable prominence of the muscularis and submucosal layers, similar to the previously imaged duodenum. Another provided thickness of this altered layer jejunum is normal at 2.1 mm

• Ileum: The ileum is empty, best seen on a provided transverse plane movie of the right kidney. Similar to the above jejunum, the muscularis and submucosal layers are prominent, no thickness measurement provided.

• Colon: The colon contains gas and shadowing content (compatible with feces). Colonic wall thickness is not provided but sonographic layering is normal. A normal ileocolic junction is imaged.

• Left kidney: The left kidney has a subjectively bright cortex, mild loss of corticomedullary distinction due to hyperechoic nature of the medulla. The provided length measurement of 4 cm is appropriate for a cat. The left renal pelvis is non-distended, the minimal visible fluid in the pelvis considered within normal range. The mesenteric fat surrounding the kidney is subjectively, but mildly bright.

• Right kidney: The right kidney is normal in echogenicity but also has decreased corticomedullary distinction due to hyperechoic nature of the medulla. The provided length measurement of 4.5 cm is slightly large for a cat, particularly a female cat. The right renal pelvis is non-distended similar in minimal fluid content to the left. The fat surrounding the renal cortex is also mildly bright, similar to the left

• Left adrenal: The left adrenal gland is normal in size and shape, measuring 3.6 mm thick.

• Right adrenal: No images are provided.

• Urinary bladder: The urinary bladder is greatly distended with anechoic fluid. The urinary bladder wall is normal in thickness.

• Lymph nodes: No images are provided.

• Peritoneal space: Mild anechoic free fluid is noted cranial to the liver, in the gallbladder fossa, adjacent to the spleen, neck of the urinary bladder and the descending colon. The mesentery is resultingly bright.

CONCLUSIONS:

• The slightly large size of the right kidney may be secondary to acute nephritis, interstitial or glomerulonephritis more likely than pyelonephritis. The loss of corticomedullary definition indicates a probable chronic component, with diffuse infiltration of the kidney, such as with amyloid, inflammation associated with feline infectious peritonitis, or lymphoma considered less likely, given the relative preservation of architecture.

• The abdominal free fluid is likely secondary to renal failure and fluid retention, recent aggressive intravenous fluid administration, or both.

Requested By:
Dr. LILIA BUSH
VEG MIAMI
2707 BISCAYNE BLVD
Miami, FL 33137
P: 786-539-0339
F:
F2:
re: Oreo Wang (3o93nl6c)

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9200 SE Sunnybrook Blvd., Suite 460
Clackamas, OR 97015

• The small intestinal wall changes suggest chronic enteropathy, either inflammatory bowel disease or small cell lymphoma, with fungal enteritis considered much less likely, but possible if the patient is indoor/outdoor in nature. The latter two differential diagnoses would usually be associated with mesenteric lymphadenopathy, which cannot be confirmed or refuted on this study. 1 image of an upper limits of normal fluid-filled jejunum with additional normal loops could indicate early mechanical small intestinal obstruction, or peristalsis only.

• The slightly hypoechoic left pancreas is suspected relative to the bright mesenteric fat, from the abdominal free fluid, rather than indicative of acute pancreatitis.

RECOMMENDATIONS:

Reexamination of the patient near the root of the mesentery to assess for the presence or absence of significant lymphadenopathy could be considered to increase or decrease the suspicion of small cell lymphoma within the small intestines (as previously described, fungal enteritis unlikely.) Correction of the patient's hyperkalemia is strongly suggested, particularly because of the patient's bradycardia, will likely be assisted by correction of acidosis and administration of glucose. Continued aggressive diuresis is also warranted, but with close watch on the patient's respirations, in case of fluid overload and secondary congestive heart failure. If not performed recently, a current feline retrovirus status and tickborne disease testing may be useful to identify potential causes of glomerulonephritis in this patient. Leptospirosis testing with appropriate serovars for your region could also be considered. If the patient continues to vomit with decreasing azotemia and there is concern for mechanical small intestinal obstruction, submission of abdominal radiographs are more completely and definitively evaluated by an outside party, for obstruction. Consult with an internal medicine specialist and/or criticalist is also warranted, particularly if the patient becomes fluid overloaded, or azotemia does not respond to the fluid therapy.

Follow-up on cases is always appreciated! If you have any questions, please do not hesitate to contact us at 1-800-726-1212 (urgent questions) or telemedicinesupport@idexx.com (non-urgent questions) to discuss the case. This contact information is for veterinary professional use only, please. Thank you for choosing IDEXX Telemedicine Consultants!

Elizabeth Ballegeer, DVM, Dip ACVR
Radiologist

Requested By:
Dr. LILIA BUSH
VEG MIAMI
2707 BISCAYNE BLVD
Miami, FL 33137
P: 786-539-0339
F:
F2:
re: Oreo Wang (3o93nl6c)

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Report provided by:
IDEXX Telemedicine Consultants
1-800-726-1212
9200 SE Sunnybrook Blvd., Suite 460
Clackamas, OR 97015

Veterinary Emergency Group
2707 Biscayne Blvd, Miami
Florida, 33137-4520, United States

Powered by ZoetisDx

Patient: - oreo 11:50pm, Cat, Unknown

Owner:

Requesting Doctor: None

i-STAT Alinity v - CG8+

Test	03/04/26 11:54 PM i-STAT	Reference Interval	Graph
K	**8.2** mEq/L	3.4 - 4.9	High
Na	**125** mEq/L	145 - 157	Low
iCa	**1.21** mmol/L	1.04 - 1.44	
pH	**6.999**	7.28 - 7.46	Low
Glu	**67** mg/dL	70 - 161	Low
TCO2	**7** mEq/L	16 - 24	Low
pO2	**72** mmHg	90 - 110	Low
pCO2	**26.1** mmHg	25 - 42	
BE	**-25** mEq/L	-11 - -1	
HCO3	**6.4** mEq/L	15 - 24	Low
sO2	**84** %	95 - 100	
Hb	**8.5** g/dL	9 - 17	Low
Hct	**25** %PCV	26 - 50	Low
Calculated pH	**6.991**		
Calculated PO2	**75** mmHg		
Calculated PCO2	**26.9** mmHg		

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Pittsburgh, PA 15201
Phone: (844) 633-3322
Fax: (570) 285-5974
www.criticalconsultsdvm.com

Date/Time of Consultation: 3/5/2026 5:10 EDT
Referring Veterinarian: Dr. Zackular
Hospital Name: VEG - Miami
Hospital Phone Number: 786-539-0339

Patient Name/Last Name: Oreo Wang
Patient ID Number: 3O93NL6C
Signalment: 7y F/S Ragdoll
Weight (in kg): 5.38

Working Diagnosis:
Severe azotemia, severe hyperkalemia with ECG changes and bradycardia; severe metabolic acidosis
History:
Oreo is a 7-year-old spayed female Ragdoll who initially presented to her primary veterinarian earlier this week for lethargy and hematemesis. She was managed with outpatient supportive care but refused oral medications at home. She returned to the pDVM on Wednesday for progressive lethargy and inappetence. Bloodwork revealed severe azotemia, hyperkalemia (K 8.7), and hypocalcemia (Ca 7.2). She was transferred to VEG Miami, arriving at 17:30.
Physical Exam Findings:
T 97.4°F (hypothermic), HR 130 with gallop rhythm on initial assessment (currently 98–120), quiet but alert and responsive, still ambulatory. Occasional VPCs noted on monitoring. BP 148/110 mmHg (164 mmHg on recheck).
Clinicopathologic Findings:

Parameter	pDVM	VEG (17:30)
Creatinine	High (OOR)	17.6
BUN	High (OOR)	High (OOR)
Phosphorus	High	
Calcium	7.2	
Sodium	133	
Potassium	8.7	9.0 → 8.2 (post-tx)
Chloride	97	
SDMA	43	
BG	166	
T. Bilirubin	1.9	
HCT	38%	
WBC	9.41	
PLT	362	
pH		6.9
pCO_2		26.1
HCO_3^-		6.4
BE		-25

USG 1.018 with proteinuria and glucosuria. Stress leukogram.

Radiographic/Ultrasound Findings:
Radiographs (pDVM): ingesta in small bowel, no evidence of obstruction, no perinephric fluid. No renal pelvis dilation or pyelectasia. Abdominal ultrasound planned for tomorrow.

Radiographs submitted for Radiologist review: ☒Yes ☐ No

Data Submitted to Critical Consults:
- ☒ Verbally Reported
- ☐ Medical Records
- ☐ Labwork (please list):
- ☐ Imaging (please list):

Current Treatment:
1. Albuterol (inhaled, x2 puffs, repeated)
2. IV dextrose boluses
3. Maropitant (Cerenia)
4. Ondansetron
5. Half-strength saline at half maintenance + LRS for the remainder
6. Regular insulin with dextrose

Recommended Diagnostics:
1. Urine culture
2. Abdominal ultrasound
3. Recheck blood gas and electrolytes in 2–4 hours – need to confirm potassium is trending down and assess response to bicarbonate therapy. The initial pH of 6.9 with BE -25 is severe and requires reassessment
4. Continuous ECG monitoring to assess for hyperkalemia-associated conduction disturbances (peaked T waves, widened QRS, bradycardia) and resolution of ventricular ectopy as potassium normalizes

Recommended Treatments:
1. Calcium gluconate 100 mg/kg IV slow over 10–15 minutes with continuous ECG monitoring – this is cardioprotective and should be given immediately given the life-threatening hyperkalemia (K 9.0) with ventricular arrhythmia. Calcium gluconate does not lower potassium but stabilizes the myocardial cell membrane against the conduction disturbances caused by hyperkalemia
2. Continue insulin-dextrose protocol for potassium shifting – regular insulin 0.25–0.5 U/kg IV with 1–2 g dextrose per unit of insulin to prevent hypoglycemia. Monitor BG q1h
3. Sodium bicarbonate therapy per the following calculation (see detailed calculation in Comments/Notes section)
    a. Dilute 14 mL of 8.4% $NaHCO_3$ with 70 mL of sterile water or D5W and administer at 14mL/h for six hours. Recheck blood gas when the infusion ends. If targets are not met, recalculate and give half of the new deficit.
    b. Targets: HCO_3^- 12–15 mEq/L, pH 7.15–7.20, Base Excess −7 to −5 mEq/L. We are not aiming for full correction, just improvement to a level more compatible with survival.
4. Enrofloxacin 5 mg/kg IV q24h – empirical antibiotic coverage pending urine culture results. The combination of severe azotemia with isosthenuria, glucosuria, and proteinuria warrants empirical treatment for pyelonephritis

This report represents the opinion of our consultants and should not be construed as a valid veterinarian-client-patient relationship. All recommendations should be followed at the discretion of the attending veterinarian. For additional questions or comments, please contact us at 1-844-633-3322.

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5. Continue IV fluid therapy – the current combination of half-strength saline and LRS is appropriate.
6. Continue antiemetics (maropitant and ondansetron) and monitor for ongoing hematemesis

Comments/Notes:
Oreo presents with severe azotemia (creatinine 17.6) and life-threatening hyperkalemia (K 9.0 on arrival) with associated ventricular ectopy. The immediate priority is myocardial stabilization with calcium gluconate and potassium reduction with insulin-dextrose and albuterol. The severe metabolic acidosis (pH 6.9, BE −25) is contributing to the hyperkalemia by driving transcellular potassium shifts, and addressing the acidosis with bicarbonate therapy will help lower serum potassium in addition to its direct acid-base benefits.

The underlying etiology of the severe azotemia needs to be determined. In a 7-year-old cat with acute-onset severe azotemia, the primary differentials include ureteral obstruction (bilateral or unilateral in a patient with pre-existing contralateral disease), acute kidney injury from toxin exposure (lily, NSAID, ethylene glycol), pyelonephritis, or acute-on-chronic kidney disease. The isosthenuria with proteinuria and glucosuria is notable – renal glucosuria in the absence of significant hyperglycemia (BG 166 is only mildly elevated) suggests proximal tubular damage. The abdominal ultrasound tomorrow will be critical for differentiating obstructive from non-obstructive etiologies and assessing renal architecture.

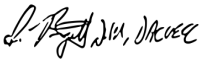
Bicarbonate Deficit Calculation:
Patient Weight: 5.38 kg
Measured HCO_3^-: 6.4 mEq/L

HCO_3^- deficit (mEq) = 0.3 × Body Weight (kg) × (24 − Measured HCO_3)
HCO_3^- deficit = 0.3 × 5.38 × 17.6 = 28.4 mEq

Administration Plan: Give HALF of this deficit (14.2 mEq), diluted 1:5 with sterile water or D5W, over six hours. Dilute 14.2 mL of 8.4% $NaHCO_3$ with 71 mL of sterile water or D5W and administer at 14.2 mL/h for six hours. Recheck blood gas when the infusion ends. If targets are not met, recalculate and give half of the new deficit.

Treatment Goals: HCO_3^- 12–15 mEq/L, pH 7.15–7.20, Base Excess −7 to −5 mEq/L. We are not aiming for full correction.

Consultant Name: Armi Pigott, DVM, DACVECC

Electronic Signature: 

Current Consultation Type: One Time Consultation - $195
- One Time Consultation is a single phone consultation on a new case
- Intermediate Consultation is 2-4 separate consultations within 24 hours on a single case
- Advanced Consultation is 5+ separate consultations within 24 hours on a single case